BASELINE → TREATMENT → FOLLOW-UP

Treatment safety, NET response and follow-up

Your team books and reviews each test. Response scans and hormone or tumour markers follow an individual plan; they are not automatically required at every medicine review.

Check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver	Baseline tests	Before each 28-day cycle	Earlier tests if symptoms or dehydration	Review results before prescribing	Follow persistent abnormalities
Blood pressure / heart	Baseline BP; cardiac assessment and echo/MUGA if indicated	BP at every attendance; heart tests if indicated	Report chest pain, breathlessness or swelling	Agree management of BP or heart changes	Individual follow-up
Thyroid / urine / glucose	Thyroid tests; baseline urine assessment; glucose review	Thyroid every 12 weeks; urine/ glucose as clinically indicated	Report cold intolerance, marked fatigue or low-glucose symptoms	Review medicine effects and diabetes treatment	Follow unresolved changes
NET response	Record symptoms, disease and personal goals	Consultant-directed imaging / markers	Track symptoms and adherence	Assess benefit and treatment burden	Plan next treatment or surveillance

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00327 v5 · current Irish regimen · EMA product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Sunitinib 37.5 mg · pancreatic NET: treatment and checks

Progressive unresectable or metastatic well-differentiated pancreatic neuroendocrine tumour under NCCP 00327a.

Take the prescribed sunitinib dose once daily continuously, at the same time, consistently with or without food. The 28-day cycle is a review period, with no planned capsule-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00327	Baseline tests	Before each 28-day cycle	Earlier tests if symptoms or dehydration	Review results before prescribing	Follow persistent abnormalities
Blood pressure / heart NCCP 00327	Baseline BP; cardiac assessment and echo/MUGA if indicated	BP at every attendance; heart tests if indicated	Report chest pain, breathlessness or swelling	Agree management of BP or heart changes	Individual follow-up
Thyroid / urine / glucose NCCP 00327	Thyroid tests; baseline urine assessment; glucose review	Thyroid every 12 weeks; urine/ glucose as clinically indicated	Report cold intolerance, marked fatigue or low-glucose symptoms	Review medicine effects and diabetes treatment	Follow unresolved changes
NET response NCCP 00327	Record symptoms, disease and personal goals	Consultant-directed imaging / markers	Track symptoms and adherence	Assess benefit and treatment burden	Plan next treatment or surveillance

What each check is for

1 Continuous daily sunitinib

28-day review

Sunitinib is a daily targeted capsule used to help control a pancreatic NET that cannot be removed or has spread and is progressing. The team reviews control, side effects and your priorities.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00327

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37